Who MediChain helps, and how we will know

The problem, in South African evidence

Finding	Source
Missing medical records "negatively affected timely and effective healthcare service delivery"; patients waited longer and some were "treated without medical history". Nurses and doctors were "unable to assist patients or treat them immediately".	Survey of 40 public hospitals, Limpopo [1]
"Numerous South African public health facilities fail to meet their record-keeping obligations as required by law", undermining patients' medical-negligence claims.	Review of South African case law, 2024 [2]
At Khayelitsha Hospital, stacks of un-scanned paper folders left months of emergency-centre records unavailable on the hospital's electronic system.	Single-hospital study, emergency centre, 2012–2013 [3]
National policy calls for person-centred digital health, a complete electronic health record and a common patient identifier.	National Digital Health Strategy 2019–2024 [4]

These studies describe the problem; they are not MediChain outcomes. The Khayelitsha study is historical and covers one hospital, so it is not a national rate.

Who benefits

Who	What changes
Patients	Carry their emergency summary (QR or NFC); see every access to their record; control who on the care team can open it; read their notes, results, medicines and messages.
Paramedics and emergency staff	See blood type, allergies, medicines, conditions and emergency contacts in seconds, on an approved device, with the access recorded.
Clinicians and nurses	One shared record for notes, orders, results and handovers; notifications only about their own patients.
Pharmacists and laboratories	Prescriptions and specimens tied to the patient and the ordering clinician; dispensing and results visible to the care team.
Facilities and regulators	A tamper-evident record of every access, which supports POPIA accountability and disputes about care.

What a pilot will measure

- **Time to critical information:** seconds from first contact to a responder seeing allergies and medicines, compared with the current folder or phone-call route.
- **Completeness at handover:** share of emergency handovers with allergies, medicines and contacts recorded.
- **Patient understanding:** whether patients can say who opened their record, and why, after using access history.
- **Adoption:** active staff per shift, and workflows completed without paper fallback.

Every number in a pilot report will be measured; MediChain makes no claim about mortality or error reduction.

Sources

1. Marutha, N. S. & Ngoepe, M. (2017). The role of medical records in the provision of public healthcare services in the Limpopo province of South Africa. *South African Journal of Information Management*, 19(1), a873. https://doi.org/10.4102/sajim.v19i1.873
2. Kolawole, O. (2024). Non-compliant health record-keeping in South Africa: Judicial responses, and progress towards a digital remedy. *The African Journal of Information and Communication*, (33), 1–19. https://doi.org/10.23962/ajic.i33.18892
3. Ohuabunwa, E. U., Sun, J., Jubanyik, K. J. & Wallis, L. A. (2016). Electronic medical records in low to middle income countries: the case of Khayelitsha Hospital, South Africa. *African Journal of Emergency Medicine*, 6(1), 38–43. https://doi.org/10.1016/j.afjem.2015.06.003
4. National Department of Health (2019). *National Digital Health Strategy for South Africa 2019–2024*. https://knowledgehub.health.gov.za/system/files/elibdownloads/2023-04/national-digital-strategy-for-south-africa-2019-2024-b.pdf